

August 12, 2026

UnitedHealthcare
Provider Appeals
P.O. Box 30432
Salt Lake City, UT 84130

RE: Formal Appeal of Claim Denial — Malik Ward (MRN MUSC8495959), Claim MUSC-20250916-C36D-1

Patient	Malik Ward (DOB 1957-04-24)
MRN	MUSC8495959
Member ID / Group	UHC725804182 / GRP-83998
Claim number	MUSC-20250916-C36D-1
Date of service	2025-09-16
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$392.73
Denial code	CARC 197 / RARC N54 — Precertification/authorization/notification/pre-treatment absent.
Appeal deadline	2026-11-01

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this formal first-level appeal on behalf of Malik Ward (Member ID: UHC725804182, Group: GRP-83998) regarding the denial of Claim MUSC-20250916-C36D-1 for services rendered on September 16, 2025, by James H. Ferrell, MD (NPI 1992837465) at MUSC Health University Medical Center. The claim was denied under CARC 197 ("Precertification/authorization/notification/pre-treatment absent") and RARC N54 ("Claim information is inconsistent with pre-certified/authorized services"), with the payer remark stating "No prior authorization on file for the date of service billed." MUSC Health respectfully contends that this denial is inconsistent with the nature of the service provided and requests full reimbursement of the billed amount of \$392.73.

CLINICAL BACKGROUND

Mr. Ward is a 69-year-old male with an established and complex medical history that includes active atrial fibrillation, managed continuously since May 2010, and obesity (BMI 30.55 kg/m²). His cardiac history is significant, encompassing multiple electrical cardioversions and catheter ablation procedures over the course of several years. He has been maintained on a long-standing medication regimen that includes Verapamil Hydrochloride, Warfarin Sodium, and Digoxin, all active since 2010, reflecting the ongoing and medically intensive nature of his cardiac management. On September 16, 2025, Mr. Ward presented to MUSC Health University Medical Center as an established outpatient patient and was evaluated by Dr. Ferrell. The encounter was billed as CPT 99214, an office or outpatient visit for an established patient of moderate complexity, with a primary diagnosis of acute upper respiratory infection (J06.9) and a secondary diagnosis of obesity (E66.9).

BASIS FOR APPEAL

CPT code 99214 represents a standard evaluation and management service for an established patient in an outpatient setting. Evaluation and management visits of this type, particularly for established patients presenting with acute conditions such as an upper respiratory infection, are widely recognized as routine outpatient services that do not require prior authorization under most commercial health plan benefit structures. UnitedHealthcare's denial rests solely on the assertion that no prior authorization was on file; however, MUSC Health's position is that no prior authorization was required for this category of service. An established-patient office visit billed under CPT 99214 is not a procedure, surgery, advanced imaging study, or other service type that customarily triggers a prior authorization requirement under standard commercial HMO plan designs.

Furthermore, Mr. Ward's active diagnoses — including atrial fibrillation requiring ongoing pharmacologic management with anticoagulation and rate-control agents — make periodic outpatient evaluation not only appropriate but medically necessary. The complexity of his medication regimen alone, which includes Warfarin Sodium and Digoxin, necessitates regular clinical oversight to ensure therapeutic safety and efficacy. The moderate complexity level assigned to this visit is consistent with the patient's documented comorbidities and the clinical judgment required to manage an acute illness in the context of his underlying conditions. The denial under RARC N54 further implies a discrepancy with pre-authorized services; however, if no authorization was sought because none was required, this remark code is inapplicable on its face.

REQUESTED ACTION

MUSC Health respectfully requests that UnitedHealthcare conduct a thorough review of this appeal and confirm whether CPT 99214 rendered in an established outpatient setting is subject to prior authorization requirements under Mr. Ward's UnitedHealthcare HMO plan (Group: GRP-83998, coverage period January 1, 2024 through December 31, 2026). If prior authorization is not required for this service type, we request that the denial be reversed and that payment be issued at the contracted rate, with the allowed amount of \$283.27 applied accordingly. Should UnitedHealthcare maintain that authorization was required, we request that the specific benefit provision or clinical policy mandating such authorization be provided in writing so that MUSC Health may respond with additional documentation. This appeal is submitted within the applicable appeal deadline of November 1, 2026, via the required electronic reconsideration process through the Claims and Payments portal.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record